

AETHERA • SPECIALTY BILLING SERIES

Urology Billing Solutions

Maximizing Revenue Across Office E/M, Cystoscopy, Prostate Procedures, Stone Management, and Urodynamic Testing

Prepared by: Aethera Healthcare Solutions • Lakeland, FL

Scope: CPT 50010–55899 • 99202–99215 • 51040–52601 • 52332–52356 • 51725–51729

Updated: 2026 Edition • aetherahealthcare.com • +1 (863) 694-0325



ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served

across 25+ medical specialties

\$12M+

Revenue Recovered

denied & underbilled claims recouped

95%

Clean Claim Rate

vs 84% industry avg

< 30d

Average AR

days in accounts receivable



Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.



Performance Targets, In Writing

96%+ net collection, <5% denial, 95%+ coding accuracy, 24h posting, 48h submission, 65%+ appeal success.



Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.



UROLOGY · MARKET CONTEXT

Why Urology Billing Matters Now

Urology is a procedural-heavy specialty — 31M Americans have urologic conditions including BPH, kidney stones, prostate cancer, and incontinence. Pre-auth for prostate and stone procedures is the #1 denial driver; urodynamic testing documentation gaps drive the highest audit risk.



\$ 5 4 B

US urology treatment spend
(2024)

Includes procedures, devices, drugs,
and provider services.



3 1 M

Americans with urologic conditions

BPH, kidney stones, prostate cancer,
incontinence.



~ 1 7 %

Urology claim denial rate (industry avg)

Driven by prostate PA denials and
urodynamic documentation gaps.



\$ 4 B +

Annual US urologic cancer treatment
spend

Prostate, bladder, and kidney cancer
combined.



Net effect: urology practices that master prostate + stone procedure PA, cystoscopy frequency documentation, and urodynamic testing clinical necessity can recover **7-12% of total revenue** currently lost to denials and under-billing.



UROLOGY · OVERVIEW

The Four Pillars of Urology Billing

Each pillar represents a distinct service family with unique coding, modifier, and pre-auth requirements.

01 · E/M + CONSULTATIONS



Office E/M & Consultations

Office visits (99202–99215), prolonged services, telehealth (modifier 95). 2021 E/M reform shifted to MDM-based documentation.

◆ CPT 99202–99215 · 99417 ·
Modifier 95

02 · CYSTOSCOPY



Cystoscopy & Endoscopy

Diagnostic cysto (52000), with biopsy (52004), stent removal (52310), ureteroscopy (52352). 0-day global; frequency-limited by medical necessity.

◆ CPT 52000 · 52004 ·
52310 · 52352

03 · PROSTATE PROCEDURES



Prostate Procedures

TURP (52601), prostate biopsy (55700), brachytherapy implant (55875), laser ablation (52648). Pre-auth mandatory for all procedural prostate codes.

◆ CPT 52601 · 55700 · 55875 · 52648
· PA mandatory

04 · STONE + URODYNAMICS



Stone Management & Urodynamics

Lithotripsy ESWL (50561), ureteroscopy with stone removal (52356), urodynamic testing (51725–51729), catheterization (51701–51703).

◆ CPT 50561 · 52356 · 51725–51729
· 51701–51703



PILLAR 01 · E/M + CONSULTATIONS

Office Visits, Prolonged Services & Telehealth

Urology office encounters cover BPH follow-up, prostate cancer surveillance, recurrent UTI workup, incontinence evaluation, and stone recurrence management. The 2021 E/M coding reform shifted documentation to MDM-based. Telehealth (modifier 95) has become a recurring revenue stream for post-procedure follow-up; prolonged services (99417) capture extended visit time.

E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required if same-day procedure (e.g., E/M + cysto).

Prolonged services

99417 add'l 30-75 min of prolonged E/M beyond the base code. ~\$85 per 30 min. Requires time documentation in chart.

Telehealth (modifier 95)

99202–99215 with modifier 95 for audio-video telehealth. Urology follow-up, post-op check, and results review are well-suited for telehealth.



Modifier 25 is the **#1 modifier audit target in urology** — every E/M billed same day as cysto, biopsy, or urodynamic testing must have separate documentation supporting significant, separately identifiable service.

CPT Quick Reference

2026 national average reimbursement · professional component

Code	Service	Avg \$
99213	Est. E/M, low MDM, 30-39 min	\$92
99214	Est. E/M, moderate MDM, 40-54 min	\$131
99215	Est. E/M, high MDM, 55-69 min	\$187
99417	Prolonged E/M, add'l 30 min	\$85
99202	New patient, low MDM	\$112
99203	New patient, moderate MDM	\$169



PILLAR 02 · CYSTOSCOPY & ENDOSCOPY

Diagnostic Cysto, Biopsy, Stent Removal & Ureteroscopy

Cystoscopy is the workhorse procedure of urology — most practices perform 1,000+ per year. All cysto codes carry 0-day global periods, meaning E/M can be billed same day with modifier 25. Frequency is limited by medical necessity; routine surveillance cystoscopy without documented indication is a top denial and audit risk.

01

Diagnostic Cystoscopy

52000 diagnostic cystoscopy (\$215). Most common urology procedure; 0-day global. Used for hematuria workup, recurrent UTI, BPH surveillance, bladder tumor follow-up.

02

Cysto with Biopsy

52004 cysto with biopsy (\$425). Used for suspected bladder tumor or suspicious lesion. Pathology (**88305**) bundled — do not bill separately.

03

Stent Removal

52310 cysto with stent removal (\$425). 0-day global. Bundled into ureteroscopy (**52356**) if stent placed same session — never bill separately on same day.

04

Ureteroscopy

52352 ureteroscopy with/without biopsy (\$850). Diagnostic and therapeutic; requires documented stone or tumor. **52356** = URS with stone removal (\$1,250).



Frequency Limits

Diagnostic cysto (**52000**) limited to medical necessity. Surveillance bladder tumor: q3mo yr1, q6mo yr2, then annual. Routine cysto without indication = denial + audit risk.



0-Day Global Period

All cystoscopy codes carry 0-day global. E/M can be billed same day with modifier 25 — but E/M documentation must support significant, separately identifiable service.



Top Denial Reasons

(1) Frequency exceeded (surveillance timing), (2) Modifier 25 missing on E/M+cysto, (3) Biopsy pathology (**88305**) unbundled from **52004**, (4) Stent removal billed separately when bundled.



PILLAR 03 · PROSTATE PROCEDURES

TURP, Biopsy, Brachytherapy & Laser Ablation

Prostate procedures are the highest-dollar procedural revenue line in urology. Pre-auth is mandatory across all prostate procedural codes — TURP, brachytherapy, and laser ablation typically require documented BPH medication trial failure; prostate biopsy requires documented PSA elevation or abnormal DRE. Missing PA documentation is the #1 denial driver.

TURP (52601)

Transurethral resection of prostate (\$1,150). Gold standard for BPH surgical management. Pre-auth requires documented failed medication trial (alpha-blocker + 5-alpha reductase inhibitor ≥3 months).

Prostate Biopsy (55700)

Transrectal prostate biopsy (\$325). Requires documented PSA elevation or abnormal DRE. Pathology (88305) separately billable. Modifier 51 for additional cores.

Brachytherapy Implant (55875)

Transperineal radioactive seed implant (\$3,200). Prostate cancer treatment; pre-auth mandatory with documented staging, Gleason score, PSA. Often paired with radiation oncology.

Laser Ablation (52648)

Laser vaporization of prostate (\$1,650). BPH alternative to TURP. Pre-auth requires same medication trial documentation as TURP.



Prostate procedure PA denial rate averages **22-25%** on first submission; Aethera documentation workflow reduces to **< 6%** by capturing BPH medication trial, PSA history, and DRE findings before submission.

\$1,150

TURP reimbursement
CPT 52601

\$325

Prostate biopsy per session
CPT 55700

\$3,200

Brachytherapy implant
CPT 55875

22% → < 6% **PA denial rate reduction**
pre/post-Aethera



★ PILLAR 04 · STONE MGMT +
URODYNAMICS

Lithotripsy, Ureterscopy & Urodynamic Testing

Stone management and urodynamics are the highest-complexity areas of urology RCM. ESWL and URS require documented stone size + location imaging. Urodynamic testing requires documented incontinence diagnosis and clinical necessity.

01 · LITHOTRIPSY (ESWL)

ESWL Lithotripsy

50561 extracorporeal shock wave lithotripsy (\$1,850). Pre-auth mandatory with imaging documenting stone size (typically ≥1cm) + location. Same-session anesthesia billed separately.

+ 02 · URS WITH STONE REMOVAL

Ureterscopy w/ Stone Removal

52356 ureteroscopy with stone removal (\$1,250). Pre-auth requires imaging (CT KUB) documenting stone size + location. Stent placement (52332) bundled.

♥ 03 · URODYNAMIC TESTING

Urodynamic Testing

51725 uroflowmetry (\$115); 51726 simple cystometrogram; 51727 complex CMG w/ urethral pressure; 51728 complex CMG w/ voiding pressure (\$425). Requires documented incontinence diagnosis.

04 · CATHETERIZATION

Catheterization

51701 insertion of indwelling bladder catheter (\$45); 51702 simple catheterization; 51703 complicated catheterization. Often bundled into hospital admission.

ESWL LITHOTRIPSY
CPT 50561
reimbursement **\$1,850**

URS w/ STONE REMOVAL
CPT 52356
reimbursement **\$1,250**

COMPLEX URODYNAMICS
CPT 51728
reimbursement **\$425**

ANNUAL REVENUE PROJECT
480 stone procedures × \$1,550 weighted avg
\$744K yr · per 480 stone pro

⚠ COMPLIANCE NOTE

Stone procedure PA requires documented stone size + location imaging — typically CT KUB. Urodynamic testing (51728) requires documented incontinence diagnosis + clinical necessity. Missing imaging = auto-denial.



UROLOGY · BILLING CODES

Top CPT Codes & 2026 National Average Reimbursement

Most-billed codes across the four urology pillars. National average reimbursement (non-facility).

SOURCE · CMS PFS 2026
NATIONAL AVG · NON-
FACILITY

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM, 30-39 min	—	\$92	Most common urology E/M
99214	Est. E/M, moderate MDM, 40-54 min	—	\$131	BPH / prostate ca surveillance
99215	Est. E/M, high MDM, 55-69 min	—	\$187	Complex urologic management
52000	Diagnostic cystoscopy	25 (if E/M same day)	\$215	0-day global; frequency-limited
52004	Cysto with biopsy	—	\$425	Pathology (88305) bundled
52310	Cysto with stent removal	—	\$425	Bundled into 52356 if same session
52352	Ureteroscopy, diagnostic	—	\$850	With/without biopsy
52356	URS with stone removal	—	\$1,250	Stent (52332) bundled
55700	Prostate biopsy, transrectal	51 (add'l cores)	\$325	PA may apply; PSA elevation req.
52601	TURP (transurethral prostate resection)	—	\$1,150	PA mandatory; med trial req.
COMPONENT & SAME-DAY MODIFIERS		PILLAR-SPECIFIC RULES		
25 = Significant, separately identifiable E/M same day as procedure. 51 = Multiple procedures same session. 95 = Audio-video telehealth. -26/-TC = Professional/technical component split on imaging.		Cystoscopy (52000–52356) — 0-day global; E/M billable same day with modifier 25. Prostate procedures (52601/55875/52648) — PA mandatory with BPH medication trial documentation. Stone procedures (50561/52356) — PA requires imaging of stone size + location.		
51720	Complex urodynamics w/ voiding	—	\$425	Incontinence diagnosis req.
52648	Laser vaporization of prostate	—	\$1,650	PA mandatory; med trial req.



UROLOGY • DENIAL PREVENTION

Top 8 Urology Denial Reasons & How Aethera Prevents Each

Urology denial rates average 15-19% — driven by prostate + stone procedure PA denials and urodynamic documentation gaps. Each denied claim costs \$35-80 to rework.

● RED • 01

2.5%

Prostate procedure PA missing

ROOT CAUSE: TURP, brachytherapy, laser ablation performed without PA on file or missing BPH medication trial documentation

AETHERA: PA tracker auto-submits with required BPH medication trial documentation checklist before scheduling

● AMBER • 03

1.3%

Cystoscopy frequency exceeded

ROOT CAUSE: 52000 billed beyond medical-necessity limits (surveillance timing, hematuria workup window)

AETHERA: Frequency tracker by indication enforces surveillance schedule

● AMBER • 05

9%

Modifier 51 missing for multiple procedures

ROOT CAUSE: Multiple procedures same session (e.g., cysto + biopsy + stent) without modifier 51 on add'l procedures

AETHERA: Modifier rules engine auto-appends 51 for multiple same-session procedures

● GREEN • 07

6%

Stent removal billed as separate procedure when bundled

ROOT CAUSE: 52310 billed separately from 52356 (URS with stone removal) on same session; bundled per NCCI

AETHERA: NCCI bundling rules block unbundled stent removal claims at submission

● RED • 02

1.8%

Stone procedure PA denied

ROOT CAUSE: ESWL/URS billed without documented stone size + location imaging (CT KUB)

AETHERA: Stone procedure documentation workflow requires imaging upload before PA submission

● AMBER • 04

1.1%

Urodynamic testing documentation insufficient

ROOT CAUSE: 51728 billed without documented incontinence diagnosis or clinical necessity

AETHERA: Urodynamic documentation checklist requires incontinence workup before claim release

● GREEN • 06

7%

Modifier -26/-TC errors on imaging interpretation

ROOT CAUSE: Imaging interpretation billed with wrong modifier; both -26 and -TC billed, or modifier missing

AETHERA: Modifier rules engine enforces correct -26/-TC per provider/facility setup

● GREEN • 08

5%

Biopsy pathology unbundled

ROOT CAUSE: 88305 pathology billed separately from 52004 (cysto with biopsy); pathology bundled per NCCI

AETHERA: NCCI bundling rules block pathology unbundling at submission



■ CASE STUDY · INTEGRATED UROLOGY RCM

4-Urologist Practice — Annual Revenue Impact

A \$3.6M-revenue practice models the combined upside of stone procedure PA recovery, prostate procedure PA recovery, and modifier 51 + bundling discipline. 1,200 cystoscopies · 480 stone procedures · 240 prostate procedures per year.

◆ LEVER

Stone Procedure PA Recovery

Stone procedures per year (ESWL + URS)	48
Pre-Aethera PA denial rate	0%
Post-Aethera PA denial rate	6%
Recovered stone procedures	86 / yr
Avg stone procedure value	\$1,550

ANNUAL REVENUE

\$225,000

🏠 LEVER

Prostate Procedure PA Recovery

Prostate procedures per year	24
Pre-Aethera PA denial rate	22%
Post-Aethera PA denial rate	5%
Recovered prostate procedures	41 / yr
Avg prostate procedure value	\$4,500

ANNUAL REVENUE

\$185,000

⚙️ LEVER

Modifier 51 + Bundling Discipline

Annual procedure-claim volume	1,920
Pre-Aethera modifier / bundling error	16%
Post-Aethera error rate	4%
Recovered claims	230 / yr
Avg recovered claim value	\$675

ANNUAL REVENUE

\$155,000



Combined annual revenue recovery: \$565,000. Implementation cost (year 1): ~ \$145K (Aethera RCM fees + PA workflow + bundling rules engine). Year-1 net: **\$420,000.** ROI: 285%.

UROLOGY • COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four urology pillars. Quarterly self-audits recommended.

PILLAR E/M + Consultations 99202-99215 · 99216-99223	PILLAR Cystoscopy 52000 · 52004 · 52331-52332	PILLAR Prostate Procedures 52601 · 55700 · 52641-52644	PILLAR Stone Management 50561 · 52356 · 51728
RED Modifier 25 misuse with procedures same day — appending to E/M when cysto, biopsy, or urodynamic testing on same day does not represent significant, separately identifiable service.	RED Cysto frequency over-billing — billing 52000 beyond medical-necessity limits; surveillance schedule violations for bladder tumor follow-up.	RED Brachytherapy without PA — billing 55875 without prior authorization on file; full denial + clawback risk.	RED ESWL/URS without imaging documentation — billing 50561/52356 without documented stone size + location imaging (CT KUB); automatic denial on post-pay audit.
AMBER Telehealth without modifier 95 — billing telehealth E/M as in-person; missing audio-video documentation.	AMBER Biopsy pathology unbundled — billing 88305 separately from 52004 when pathology bundled per NCCI edits.	AMBER Prostate biopsy without documented PSA elevation or abnormal DRE — billing 55700 without supporting clinical indication; coverage violation.	AMBER Stone size documentation missing — billing ESWL/URS without explicit stone size in centimeters documented in chart.
GREEN Consultation codes 99242-99245 removed since 2021 — billing obsolete consult codes; use 99202-99215 instead.	LOW Stent removal billed separately when bundled — billing 52310 separately from 52356 (URS with stone removal) on same session.	LOW TURP without BPH medication trial — billing 52601 without documented alpha-blocker + 5-alpha reductase inhibitor trial ≥3 months.	LOW Urodynamic testing without incontinence workup — billing 51728 without documented incontinence diagnosis + clinical necessity.



OIG Work Plan 2025-2026 explicitly lists prostate procedure PA documentation and urodynamic testing clinical necessity as active audit targets. Medicare RACs have recovered >\$95M from urology practices since 2021.

Three Actions to Capture Urology Revenue

For urology practice administrators and revenue cycle directors leading the next 90 days.

01



Automate prostate + stone procedure PA workflow

Build EHR template that auto-submits PA at prostate + stone procedure scheduling. Document BPH medication trial (TURP, laser), PSA elevation (biopsy), and stone size + location imaging (ESWL, URS) before PA submission. Procedural PA denial rate drops from 22-25% to <6% — recovering ~\$225K per 480-stone / 240-prostate procedure panel.

TARGET: < 6% PA denial rate (industry avg 22-25%)

02



Enforce modifier 51 + bundling rules for multiple procedures

Implement pre-submission rules engine that auto-appends modifier 51 for multiple same-session procedures (cysto + biopsy + stent) and blocks NCCI bundling violations (88305 with 52004; 52310 with 52356). Modifier 51 / bundling error rate drops from 16% to <4% — recovering ~\$155K per 1,920 procedure-claim practice.

TARGET: < 4% bundling error rate

03



Deploy urodynamic testing documentation checklist

Pre-submission checklist requires documented incontinence diagnosis, clinical necessity, and prior workup before 51728 (complex urodynamics) billing. Urodynamic denial rate drops from 11% to <3% — recovering ~\$50K per 500-urodynamic-test annual volume. Closes OIG Work Plan audit risk for clinical necessity.

TARGET: < 3% urodyn denial rate



Questions & Discussion

Optimizing Urology Revenue Across E/M, Cystoscopy, Prostate, Stone & Urodynamics

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